Clinical Practice Guidelines: Acute Myocardial Infarction Management

Section 1.1: STEMI Emergency Revascularization Window

For patients presenting with ST-Elevation Myocardial Infarction STEMI primary percutaneous coronary intervention PCI is the preferred strategy when performed within 90 minutes of first medical contact FMC time.

If transfer to a PCI center exceeds 120 minutes intravenous fibrinolysis must be initiated within 10 minutes of diagnosis.

Section 2.3: Antiplatelet Pharmacotherapy & Absolute Contraindications

All acute coronary syndrome patients should receive chewable Aspirin 162-325 mg along with Ticagrelor 180 mg loading dose or Prasugrel 60 mg loading dose.

Absolute contraindications to fibrinolytic therapy include:

1. Any prior history of intracranial hemorrhage ICH.
2. Known structural cerebral vascular lesion.
3. Active internal bleeding excluding menses.
4. Ischemic stroke within preceding 3 months.